

For the use only of a
Registered Medical
Practitioner or a Hospital
or a Laboratory

Salbutamol Pressurised
Inhalation BP
100 mcg/actuation



Asthavent CFC free Inhaler

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Asthavent CFC free Inhaler

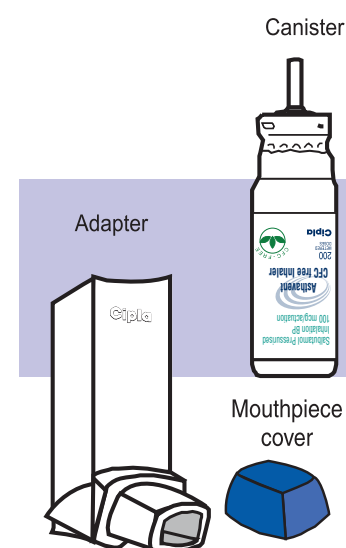
COMPOSITION

Each actuation delivers
Salbutamol Sulphate BP
equivalent to
Salbutamol.....100 mcg
Suspended in
Propellant HFA-134a.....q.s.

Cipla

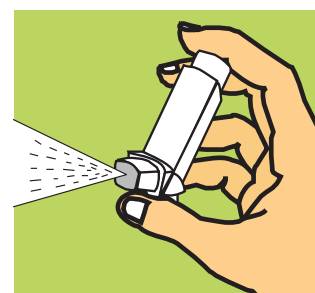
PATIENT INFORMATION LEAFLET

Parts of the Inhaler

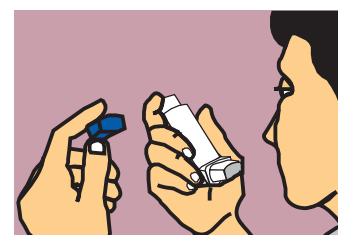


HOW TO USE YOUR INHALER CORRECTLY

Testing your inhaler
Before using your inhaler for
the first time, or if it has not
been used for a week or more,
shake it well and then "test fire"
it , i.e. release one puff into the
air.



Remove the mouthpiece cover,
and check the mouthpiece
is clean. Shake the inhaler
well before each use.



Hold the inhaler as shown
between index finger and
thumb. Breathe out gently
through your mouth and
immediately place the
mouthpiece in your mouth
between your teeth and close
your lips around it (Do not
bite it).



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Asthavent CFC free Inhaler (Salbutamol Pressurised Inhalation BP 100 mcg/actuation)

COMPOSITION

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Salbutamol Sulphate BP equivalent to
Salbutamol100 mcg
Suspended in
propellant HFA 134a.....q.s.

DOSAGE FORM

Pressurised Inhalation

Product Description

A one piece filled metal container fitted
with a valve with plastic stem and
printed with product name, batch
number and container number. On
visual examination there should be no
sign of physical damage such as dents,
bulged container bent valve & uneven
crimping.

PHARMACOLOGY

Mechanism of Action:

Salbutamol is a selective 2-adrenoceptor
agonist. At therapeutic doses, it acts on
the 2-adrenoceptors of bronchial muscle
with little or no action on 2-adrenoceptors
of cardiac muscle. Salbutamol thus
provides short acting (4-6 hours)
bronchodilatation with a fast onset
(within 5 minutes) in reversible airways
obstruction.

Pharmacokinetics

Absorption Salbutamol appears to be
absorbed from the respiratory tract
over several hours following oral
inhalation. The peak plasma salbutamol
concentrations occurs in 2.5 hours.

Metabolism and Elimination

After oral inhalation of salbutamol in
healthy adults, the elimination half-life
of unchanged drug was determined
indirectly to be 3.8 hours.

Salbutamol is extensively metabolized
in the liver, mainly to salbutamol 4' O
sulfate which has little or no -adrenergic
stimulating effect and no -adrenergic
blocking effect.

Salbutamol and its metabolites are
rapidly excreted in urine and faeces.
After oral inhalation of salbutamol in
patients with asthma, approximately
70% of a dose is excreted in urine as
unchanged drug and metabolites
within 24 hours and 80-100% within 72

hours; about 30% of the dose is
excreted in urine unchanged in 24
hours. About 10% of an inhaled dose of
salbutamol may be excreted in faeces.

INDICATIONS

Asthavent CFC-Free Inhaler provides
short-acting (4 to 6 hour) bronchodilation
with fast onset (within 5 minutes) in
reversible airway obstruction. It is
particularly suitable for the relief and
prevention of asthma symptoms. It should
be used to relieve symptoms when
they occur, and to prevent them in
those circumstances recognised by the
patient to precipitate an asthma attack
(e.g. before exercise or unavoidable
allergen exposure). It is particularly
valuable as relief medication in mild,
moderate or severe asthma, provided
that reliance on it does not delay the
introduction and use of regular inhaled
corticosteroid therapy. Relief and
prevention of bronchospasm in bronchial
asthma, chronic bronchitis, emphysema
and acute dyspnoea.

DOSAGE AND ADMINISTRATION

For relief of acute episodes of
bronchospasm:

Adults:

1 or 2 puffs as necessary.(100 or 200
micrograms).

The maximum dose is up to 8 puffs in
24 hours or up to 200 micrograms four
times daily.

Children:

Half the adult dose. 100 micrograms,
the dose may be increased to 200
micrograms if required. The maximum
dose is up to 8 puffs in 24 hours or upto
200 micrograms four times daily.

To prevent allergen- or exercise-
induced bronchospasm:

Adults:

Two inhalations (200micrograms),15
minutes prior to exercise or exposure
to allergen.

The maximum dose is 2 inhalations, up
to 4 times a day.

Children:

Half the adult dose. One inhalation (
100 micrograms), 15 minutes prior to
exercise or exposure to allergen. The
dose may be increased to 200micrograms
if required.

CONTRAINDICATIONS

Hypersensitivity to any of the
components of the formulation.

WARNINGS AND PRECAUTIONS

Paradoxical Bronchospasm

Inhaled salbutamol sulphate can produce
paradoxical bronchospasm, which may
be life-threatening. If paradoxical
bronchospasm occurs, Asthavent CFC
free Inhaler should be discontinued
immediately and alternative therapy
instituted. It should be recognized that
paradoxical bronchospasm, when
associated with inhaled formulations,
frequently occurs with the first use of a
new canister.

Cardiovascular Effects

ASTHAVENT CFC FREE INHALER ,
like all other beta-adrenergic agonists, can
produce clinically significant cardiovascular
effects in some patients as measured
by pulse rate, blood pressure, and/or
symptoms. Although such effects are
uncommon after administration of
Asthavent CFC free Inhaler at
recommended doses, if they occur, the
drug may need to be discontinued. In
addition, beta-agonists have been
reported to produce electrocardiogram
(ECG) changes, such as flattening of
the T wave, prolongation of the QTc
interval, and ST segment depression.
The clinical significance of these findings
is unknown. Therefore, **ASTHAVENT
CFC FREE INHALER** , like all
sympathomimetic amines, should be
used with caution in patients with
cardiovascular disorders, especially
coronary insufficiency, cardiac arrhythmias,
and hypertension.

Deterioration of Asthma

Asthma may deteriorate acutely over a
period of hours or chronically over
several days or longer. If the patient
needs more doses of **ASTHAVENT
CFC FREE INHALER** than usual, this
may be a marker of destabilization of
asthma and requires re-evaluation of
the patient and treatment regimen, with
special consideration to the possible
need for anti-inflammatory treatment,
e.g., corticosteroids.

Use of Anti-Inflammatory Agents

The use of beta-adrenergic agonist
bronchodilators alone may not be
adequate to control asthma in many
patients. Early consideration should be
given to adding anti-inflammatory agents,
e.g., corticosteroids, to the therapeutic
regimen.

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Immediate Hypersensitivity Reactions

Immediate hypersensitivity reactions may occur after administration of salbutamol sulphate inhalation aerosol, as demonstrated by cases of urticaria, angio-oedema, rash, bronchospasm, anaphylaxis, and oropharyngeal oedema.

Do Not Exceed Recommended Dose

Fatalities have been reported in association with excessive use of inhaled sympathomimetic drugs in patients with asthma. The exact cause of death is unknown, but cardiac arrest following an unexpected development of a severe acute asthmatic crisis and subsequent hypoxia is suspected.

Drug Interactions

Asthavent CFC free Inhaler preparations should be used with caution in patients suffering from thyrotoxicosis. Asthavent CFC free Inhaler and non-selective beta-blocking drugs such as propranolol should generally not be prescribed together. Potentially serious hypokalaemia may result from beta₂-agonist therapy. Particular caution is advised in acute severe asthma as this effect may be potentiated by concomitant treatment with xanthine derivatives, steroids, diuretics, and by hypoxia. It is recommended that serum potassium levels be monitored in such situations.

Pregnancy

Administration of the drug during pregnancy should only be considered if the expected benefit to the mother is greater than any possible risk to the foetus.

Lactation

As salbutamol is probably secreted in breast milk, its use by lactating mothers requires careful consideration.

UNDESIRABLE EFFECTS

Mild tremor, hypokalaemia, palpitations, muscle cramps, oropharyngeal irritation, and headache have been rarely reported. These usually disappear with continued treatment. Paradoxical bronchospasm may occur, in which case, the therapy should be discontinued immediately and alternative therapy instituted. Undesirable effects associated with Cardiovascular system may include myocardial ischaemia (reported spontaneously in post-marketing data, therefore frequency is regarded as unknown).

OVERDOSAGE

The preferred antidote for overdosage with salbutamol is a cardioselective β-Blocking agent. Beta-blocking drugs should be used with caution in patients with a history of bronchospasm. Hypokalaemia may occur following overdose with salbutamol. Serum potassium levels should be monitored.

SHELF-LIFE

2 years

STORAGE AND HANDLING INSTRUCTIONS

Store below 30°C. Do not freeze

PACK SIZE

Asthavent CFC Free Inhaler

Aerosol containing 200 and 300 metered doses

Registration Holder in Malaysia

CIPLA MALAYSIA SDN BHD
Suite 1101, Amcorp Tower, Amcorp Trade Centre, 18 Persiaran Barat, 46 050 Petaling Jaya, Selangor, Malaysia.

Mfd. by **CIPLA LTD.**,
L-139 to L-146, Verna Industrial Estate, Verna Goa INDIA

Date of Revision:

March 2018

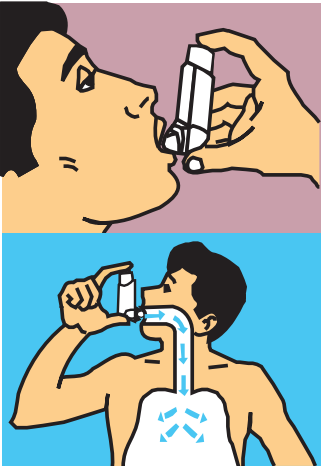
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Tilt your head slightly backwards. Start breathing in slowly through your mouth. As you breathe in, press down the canister to release one dose while continuing to breathe in steadily and deeply.



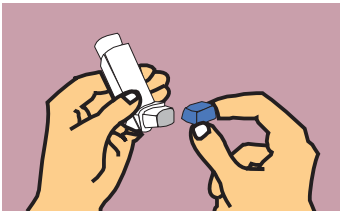
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Remove the inhaler from your mouth and hold your breath for 10 seconds, or for as long as it is comfortable. Breathe out slowly.

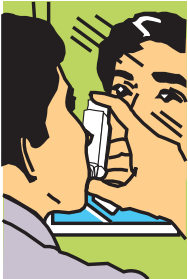


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If another dose is required, wait for at least one minute and repeat steps 2 to 4. After use, replace the mouthpiece cover.



Note :
Do not rush steps 2 and 3. It is important to breathe in slowly through your mouth just before pressing the canister. Practice in front of the mirror for the first few times. If you see 'mist' coming from the top of the inhaler or the sides of your mouth, start again from step 1.



FOR CHILDREN

Parents must assist those children who need help in using the inhaler correctly.



WARNING

Use only as directed by your doctor. If the recommended dose does not provide relief of symptoms or symptoms become worse, consult your doctor.

The metal canister is pressurised. Do not puncture, break or burn it even when empty.

Keep away from eyes.
Keep away from children.

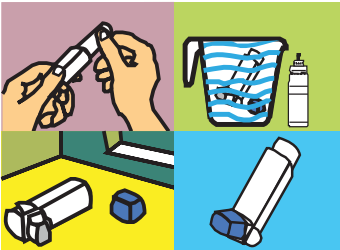
STORAGE

Store below 30°C
Do not freeze

CLEANSING

Clean your inhaler at least once a week.

1. Gently pull the metal canister out of the adapter of the inhaler. Remove the mouthpiece cover.
2. Rinse the adapter and the mouthpiece cover in warm water. Do not put the metal canister into water.
3. Shake well to remove excess water.
4. Leave to dry in warm place. Avoid excess heat.
5. Replace the canister and the mouthpiece cover correctly.



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PRESENTATION

Asthavent CFC free Inhaler

Aerosol containing 200 and 300 MD

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PACKAGING DEVELOPMENT

Product Name: Asthavent CFC Free Inhaler		Material No.: xxxxxxxx	Version: 01	Item: Leaflet	Co-ordinator: Shruti	Artist: Vasundhara	Date: 06-03-2018		
Colours: BLUE WOOL TEST VALUE 5-8 (LIGHT FASTENING DATA) Cyan Magenta Yellow Black					INK: Oil based Ink from DIC OR MICRO				
Design: Booklet		Supersedes/Reference: 21062274			Software: Illustrator CC				
Fonts: -----				Links: xxxxxx					
Actual Size: 390 x 240 mm		Size after folding: 65 x 30 mm		Pharmacode: -----		Grain Direction : Parallel to length		Screen : # __	
Material: 60 GSM Mapliitho Paper (From Andhra OR Ballarpur Paper Mill ONLY) with perforation and folded with perforated tape printed with "†Tear Here†"					Varnish:		Artwork Print Size: ☐ actual ☐ scaled		
Path: PC:/D:/Vasundhara/Malaysia/Asthavent/xxxxxxx Asthavent CFC Free Inhaler Leaflet Malaysia.ai									
• Instructions / Remark: --- • Any deviation must be brought to the notice of packaging development co-ordinator immediately. • For any clarification, please contact packaging development co-ordinator immediately. NO CHANGES IN ARTWORK SHOULD BE DONE BY THE PRINTER • The printer should verify the e-proof against the approved artwork before submitting for approval and the e-proof should have printer details .				Checked by	Artist	Cordinator	file loaded in Server	Section Head	
				Pharma Code	☐		☐		
				2D Code	☐		☐		
				QR Code	☐		☐		
				Bar Code	☐		☐		
				Artwork	☐		☐		
Spell check	☐		☐						